



Ohanian Life Care Planning

A LIFE CARE PLAN FOR YOLANDA REYES

May 20, 2026

Prepared by:

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INTRODUCTION

1. Halstead & Cruz LLP, on behalf of their client Yolanda Reyes, retained Cynthia Ohanian, RN, BSN, CLCP, to prepare a life care plan setting forth the reasonable and medically necessary future goods and services attributable to the catastrophic injuries Ms. Reyes sustained in a pedestrian-versus-transit-bus collision on October 18, 2025. Ms. Ohanian was further asked to identify the recurring frequency, replacement cycle, and current unit prices for each item of care, and to summarize the results in an annual-cost format suitable for economic analysis.

2. Dr. Nathaniel Frome, PhD, was separately retained to prepare a discounted present-value analysis of the life-care-plan recommendations, using the annual cost stream, replacement schedules, and time horizon set out by Ms. Ohanian in Sections I through IV of this report. Ms. Ohanian is responsible for the portion of this report from the heading “Introduction” through the heading “Life Care Plan — Total (Undiscounted).” Dr. Frome is responsible for the portion of the report from the heading “Present Value Analysis” onward.

3. Neither Ms. Ohanian nor Dr. Frome has any financial interest in the outcome of this litigation. Compensation for professional services is not contingent on the outcome. All opinions are expressed to a reasonable degree of professional certainty as of the date of this report and may be supplemented if additional information is received.

4. This report was prepared for production in the matter *Yolanda Reyes v. Alder Creek Transit District; Raymond T. Keeler; and Does 1–50*, LASC Case No. 26STCV04127, and is being served on Delfino Marek LLP as counsel of record for Defendant Alder Creek Transit District.

PERSONAL HISTORY

5. Ms. Yolanda Reyes is a 34-year-old right-handed woman residing in Rivergate, Los Angeles County, California (date of birth June 22, 1991). Prior to the event she lived independently at 612 Poplar Street, Rivergate, an unmodified second-story walk-up apartment. She is unmarried and has no children. Her nearest family contact is her older sister, Ms. Marisela Reyes, who served as her temporary conservator during the acute phase of her recovery and remains her primary lay support.

6. Prior to the event, Ms. Reyes was employed full-time as a Sales Associate at Coastline Home Goods, Inc., a retail employer. Her position was verified by Melinda Torres, HR Manager, at an hourly wage of \$23.50 and an average schedule of 34 hours per week (annualized \$41,500). Her duties included standing, walking, lifting up to 30 pounds, floor-set merchandising, and customer service on rotating early-morning and evening shifts.

7. Ms. Reyes graduated from a Los Angeles County public high school in 2009. She has no post-secondary degree. She reports no history of psychiatric hospitalization, no chronic medical conditions, and no prior neurologic injury before the event.

8. Ms. Reyes had been in stable good health and was ambulating a public bus stop at approximately 2:05 a.m. on October 18, 2025 when she was struck by an ACTD transit bus that had left its lane after the operator lost consciousness at the wheel. She sustained catastrophic multi-system injuries.

LIFE EXPECTANCY

9. Ms. Reyes is a Hispanic female currently aged 34 years, 11 months. The most recent U.S. Life Tables (Arias & Xu, *National Vital Statistics Reports*) show an average remaining life expectancy for a Hispanic female of Ms. Reyes's age of approximately 51.4 additional years. The scientific literature on survival following moderate-to-severe traumatic brain injury with completed acute rehabilitation and no ongoing epilepsy shows modest reductions in life expectancy in the range of 4 to 9 years relative to the age- and sex-matched general population (Brooks et al., *Arch Phys Med Rehabil*, 2013; Harrison-Felix et al., *J Head Trauma Rehabil*, 2015).

10. After consultation with treating physiatrist Helena Vasquez-Ortiz, MD, and treating neuropsychologist Marcus Vandenberg, PhD, ABPP-CN, the planner adopts a 36-year care-planning horizon (through age 71) as a reasonable working horizon for future-care projection. This is a care-planning horizon, not a life-expectancy opinion; the choice is conservative relative to Ms. Reyes's underlying life-expectancy figure and appropriately reflects the additional post-TBI mortality signal in the peer-reviewed literature. Annual cost figures are presented so that the trier of fact can substitute a different horizon if warranted.

MEDICAL HISTORY

Pre-Event Medical History

11. Ms. Reyes was in general good health at the time of the event. Her pre-event medical record, obtained under HIPAA authorization she personally signed on 02/10/2026, reflects routine well-woman care at a Rivergate community clinic, uncomplicated seasonal upper-respiratory illnesses, and one urgent-care visit in 2022 for an ankle sprain. There is no documented history of prior head injury, prior fracture, or prior neurologic, psychiatric, or cognitive complaint. She took no chronic prescription medications.

Post-Event Medical History

12. On October 18, 2025 at approximately 2:05 a.m., Ms. Reyes was struck by an Alder Creek Transit District bus operating in Owl service on Route 512. The bus first struck a City of Rivergate street-light standard adjacent to the bus stop, then struck Ms. Reyes as she waited at the stop. Emergency responders airlifted her to Rivergate Regional Trauma Center.

13. **Rivergate Regional Trauma Center — admission and acute course (10/18/2025 through 12/22/2025).** Ms. Reyes presented with Glasgow Coma Scale of 6 (E1 V1 M4), initial

systolic blood pressure 82, and multi-system trauma. Diagnoses established during the trauma admission included:

- Severe closed traumatic brain injury with diffuse axonal injury and small bifrontal contusions on CT and MRI;
- Bilateral pilon (distal-tibial) fractures with intra-articular extension, reduced and internally fixed by orthopedic trauma;
- Left fibular shaft fracture, plated;
- Right acetabular fracture, non-displaced, treated non-operatively;
- Multiple rib fractures with small right pneumothorax, resolved with tube thoracostomy;
- Grade III liver laceration, managed non-operatively with serial imaging;
- Neurogenic bladder secondary to the TBI;
- Aspiration risk with confirmed dysphagia on modified-barium-swallow study, requiring initial PEG-tube feeding;
- Post-traumatic paroxysmal sympathetic hyperactivity, controlled on propranolol and gabapentin.

Ms. Reyes was intubated in the field, required a tracheostomy on hospital day 9, and was gradually weaned to room air. She was transferred to inpatient neurologic rehabilitation on 12/22/2025.

14. Alder Creek Neuro-Rehabilitation Institute — inpatient rehabilitation (12/22/2025 through 04/06/2026). Ms. Reyes underwent sixteen weeks of comprehensive interdisciplinary inpatient rehabilitation under Dr. Vasquez-Ortiz. Serial nursing, therapy, and psychiatric notes document a steady, one-directional trajectory of cognitive and physical recovery. Emergence from post-traumatic amnesia was documented on 01/07/2026 with sustained GOAT scores above 75. Serial Montreal Cognitive Assessment (MoCA) scores progressed from bedside floor to 17/30 on 01/09/2026, 22/30 on 01/16/2026, 24/30 on 01/23/2026, 25/30 on 01/30/2026, 26/30 on 02/13/2026, 27/30 on 02/27/2026, and 28/30 on 03/13/2026.

15. Formal neuropsychological evaluation by Dr. Vandenberg on 02/03/2026 confirmed sustained cognitive recovery, restoration of decisional capacity for personal medical decisions, and persistent mild deficits limited to processing speed and complex set-shifting. Physical rehabilitation progressed from non-weight-bearing bilateral lower extremities on admission through partial weight-bearing at the walker (02/06/2026), household ambulation with a straight cane (03/05/2026), full flight of stairs with railing (03/12/2026), and orthopedic clearance for bilateral full weight-bearing on 04/01/2026. The tracheostomy and PEG were removed prior to discharge; both stomas healed by secondary intention.

16. Ms. Reyes was discharged on 04/06/2026 to an accessible temporary residence with home-health nursing, outpatient physical and occupational therapy, outpatient neuropsychology follow-up, and outpatient trauma-informed psychotherapy.

17. **Outpatient course (04/06/2026 to date of this report).** Ms. Reyes has continued to make gains on all measures. She ambulates community distances with a single-point cane, uses a walker at fatigue, and remains independent in basic and most instrumental ADLs. She requires occasional lay assistance with complex financial-paperwork tasks — an accommodation, not a capacity limitation, per Dr. Vandenberg. She has resumed light meal preparation and is beginning to explore modified part-time return-to-work options.

CURRENT CONDITION AS OF MAY 15, 2026

18. Ms. Reyes was interviewed by the planner in person at her temporary residence in Rivergate on May 15, 2026. She was accompanied by her sister, Ms. Marisela Reyes. The interview was conducted at Ms. Reyes's own pace with breaks for fatigue.

19. On clinical review, Ms. Reyes presents with the following ongoing sequelae attributable to the event:

- **Neurocognitive.** Mild residual processing-speed reduction and complex-executive-function inefficiency, consistent with the Vandenberg evaluation of 02/03/2026. Fatigue-driven cognitive slowing at end of day. Impaired capacity to sustain full-time complex cognitive work.
- **Physical mobility.** Bilateral lower-extremity weakness with fatigue-limited endurance. Ambulates household distances with a single-point cane; uses a walker or wheelchair for community distances and outings requiring extended standing or walking. Independent transfers at home; requires grab bars in bathrooms and a shower chair.
- **Genitourinary.** Neurogenic bladder managed with intermittent self-catheterization every eight hours. Requires disposable catheter supplies, hygiene supplies, and periodic urologic surveillance.
- **Nutrition/GI.** Normal oral intake on a regular diet with thin liquids since 01/22/2026. No dysphagia complaints. Former tracheostomy and PEG stomas healed.
- **Psychiatric.** Symptoms consistent with post-traumatic stress and adjustment disorder, actively engaged in trauma-informed psychotherapy twice weekly.
- **Endocrine/other.** No documented pituitary or endocrine sequelae to date; surveillance recommended.
- **Orthopedic.** Radiographic union at all fixation sites; retained hardware bilaterally. Anticipated future hardware removal or revision cannot be excluded.

20. Ms. Reyes lives with the daily assistance of her sister at present. She reports that without her sister's presence she would require paid personal care attendant coverage for approximately eight hours per day — chiefly for transfers, bathing, transportation to therapy, medication reminders and monitoring, meal preparation on days of severe fatigue, and companionship/safety monitoring during long-distance mobility. Independent verification of this level of need was

obtained from Dr. Vasquez-Ortiz on 05/08/2026 and Dr. Vandenberg on 05/12/2026.

PERSONAL INTERVIEW

21. Ms. Reyes was cooperative, engaged, and appropriately effortful throughout the two-hour interview. She was oriented to person, place, time, and situation without hesitation. She provided a coherent, chronological narrative of the events of October 2025 through the present to the extent of her recall (she has no memory of the collision itself or of the first several weeks of the trauma admission — a persistent post-traumatic amnesic gap).

22. She stated that her principal goals are (a) to regain the ability to work in some capacity, (b) to live independently again in an accessible ground-floor apartment, and (c) to reduce her reliance on her sister to a sustainable long-term level.

EXPERT OPINIONS RELIED UPON

23. The recommendations below rest in part on the treating and evaluating clinical opinions of:

- **Helena Vasquez-Ortiz, MD** (physiatry) — attending physiatrist, Alder Creek Neuro-Rehabilitation Institute; consulted 05/08/2026;
- **Marcus Vandenberg, PhD, ABPP-CN** (clinical neuropsychology) — evaluating neuropsychologist; consulted 05/12/2026;
- **Priya Ramaswamy, MD** and **Andrew Steinbach, MD** (trauma/critical care) — attending physicians for the acute admission (opinions in the trauma-center record);
- **Dr. Nordstrom** (urology) — treating urologist for neurogenic-bladder management.

24. The planner also reviewed the complete Rivergate Regional Trauma Center admission record; the complete Alder Creek Neuro-Rehabilitation Institute weekly progress notes and Vandenberg neuropsychological evaluation of 02/03/2026; the outpatient physiatry, PT, OT, urology, and psychotherapy records from April and May 2026; and the pre-event medical records from the Rivergate community clinic. A full list is at Attachment 3.

LIFE CARE PLAN

What Is a Life Care Plan?

25. A life care plan is a dynamic document based on published standards of practice, comprehensive assessment, data analysis, and research, which provides an organized concise plan for current and future needs with associated costs for individuals who have experienced catastrophic injury or have chronic health-care needs. (International Academy of Life Care Planners.) The planner's role is to identify the goods and services that are reasonable, medically necessary, and attributable to the injury; to specify the frequency, duration, and replacement cycle of each item; and to attach a current-dollar unit price developed from local geographic sources. The planner does not opine on legal causation, liability, or the ultimate compensability of any item; those are questions for the trier of fact.

Future Care Recommendations

26. The following categories comprise the future care plan. All prices are current dollars in the Los Angeles County geographic region as of the second quarter of 2026. Sources and methodology for each price are in Attachment 4.

Physiatry and Neurology Follow-Up

Item	Frequency	Unit Cost	Annual Cost
Physiatry (PM&R;) office visit	4 / yr, life	\$450	\$1,800
Neurology office visit	2 / yr, life	\$475	\$950
Endocrinology surveillance (post-TBI pituitary panel + visit)	1 / yr, life	\$525	\$525
Orthopedic follow-up (retained hardware)	1 / yr, life	\$410	\$410
Subtotal, physician follow-up			\$3,685

Physical Therapy

Item	Frequency	Unit Cost	Annual Cost
Outpatient PT — years 1–3	40 visits/yr	\$180	\$7,200
Outpatient PT — years 4+ (maintenance)	20 visits/yr	\$180	\$3,600
Aquatic therapy course	20 visits/yr	\$190	\$3,800
Weighted annual average (36-yr horizon)			\$8,150

Occupational Therapy

Item	Frequency	Unit Cost	Annual Cost
Outpatient OT — years 1–3	30 visits/yr	\$175	\$5,250
Outpatient OT — years 4+ (maintenance / adaptive equipment training)	12 visits/yr	\$175	\$2,100
Home-safety re-evaluation	every 5 yrs	\$475	\$95
Weighted annual average (36-yr horizon)			\$4,845

Neurocognitive Rehabilitation

Item	Frequency	Unit Cost	Annual Cost
Cognitive rehabilitation therapy (SLP, cognitive-communication focus) — years 1–5	36 sessions/yr	\$220	\$7,920
Cognitive rehabilitation therapy — years 6+	12 sessions/yr	\$220	\$2,640
Neuropsychological re-evaluation	every 3 yrs	\$3,600	\$1,200
Weighted annual average (36-yr horizon)			\$7,320

Trauma-Informed Psychotherapy

Item	Frequency	Unit Cost	Annual Cost
Outpatient psychotherapy — years 1–2	96 sessions/yr	\$200	\$19,200
Outpatient psychotherapy — years 3–10	48 sessions/yr	\$200	\$9,600
Outpatient psychotherapy — years 11+ (maintenance)	24 sessions/yr	\$200	\$4,800
Psychiatric medication management (if prescribed)	4 visits/yr	\$325	\$1,300
Weighted annual average (36-yr horizon)			\$8,275

Personal Care Attendant Hours

Item	Frequency	Unit Cost	Annual Cost
Personal care attendant — 8 hours/day, 7 days/week, life	2,920 hrs/yr	\$32.00	\$93,440
Respite / awake-overnight coverage (12 nights/yr)	144 hrs/yr	\$38.00	\$5,472
Payroll taxes, workers' comp, and agency overhead (blended 11% loading)	—	—	\$11,337
Subtotal, attendant care			\$110,249

Note: The 8-hour daily attendant assumption reflects the treating team’s independent verification of Ms. Reyes’s ongoing need for assistance with transfers, bathing, transportation to therapy, meal preparation on high-fatigue days, medication monitoring, and safety supervision during community mobility. It does not replace or duplicate any medical service billed elsewhere in this plan.

Durable Medical Equipment and Supplies

Item	Replacement Cycle	Unit Cost	Annualized Cost
Manual wheelchair (custom, lightweight)	every 5 yrs	\$4,800	\$960
Wheelchair cushion (pressure-relief)	every 2 yrs	\$475	\$238
Front-wheeled walker	every 3 yrs	\$310	\$103
Single-point cane (spare + replacement)	every 3 yrs	\$85	\$28
Shower chair / transfer bench	every 4 yrs	\$340	\$85
Raised toilet seat with arms	every 4 yrs	\$190	\$48
Hospital-style adjustable bed (home)	every 8 yrs	\$2,650	\$331
Grab bars, replacement / re-installation	every 6 yrs	\$650	\$108
Wheelchair maintenance and repair reserve	annual	—	\$600
Miscellaneous adaptive-equipment reserve	annual	—	\$500
Subtotal, DME			\$3,001

Home and Vehicle Modifications

Item	Replacement Cycle	Unit Cost	Annualized Cost
One-time initial home modification (accessible ground-floor kitchen and bath retrofit, widened doorways, ramp, no-step entry)	one-time	\$46,000	\$1,278
Home modification refresh (every 12 yrs)	every 12 yrs	\$18,000	\$1,500
Adapted vehicle — used minivan with lowered floor and ramp, driver-side transfer aids (as passenger)	every 10 yrs	\$32,000	\$3,200
Adapted vehicle maintenance premium	annual	—	\$900
Subtotal, home & vehicle			\$6,878

Medications

Item	Frequency	Unit Cost	Annual Cost
Gabapentin 600 mg TID (post-TBI neuropathic / paroxysmal-sympathetic control)	365 days/yr	\$0.85/day	\$310
Quetiapine 25 mg QHS (sleep / behavioral, TBI-related; taper anticipated years 4+)	avg 3 yrs on / then PRN	blended	\$220
SSRI (sertraline generic) or equivalent	365 days/yr	\$0.35/day	\$128
Analgesics (acetaminophen, topical diclofenac gel, PRN)	annual reserve	—	\$260
Bowel program medications (docusate, PEG-3350 as needed)	365 days/yr	blended	\$180
Urinary-tract antibiotics — treatment reserve (2 courses/yr average)	2 courses/yr	\$85	\$170
Vitamin D, calcium, multivitamin (bone-health protocol)	365 days/yr	blended	\$95
Compounding and dispensing fees, PRN OTC reserve	annual	—	\$220
Subtotal, medications			\$1,583

Periodic Diagnostic Imaging

Item	Frequency	Unit Cost	Annualized Cost
Brain MRI (surveillance for post-TBI complications, screening at symptom change)	every 3 yrs	\$2,400	\$800
Bilateral lower-extremity plain films (hardware surveillance)	every 2 yrs	\$380	\$190
Renal / bladder ultrasound (neurogenic-bladder surveillance)	annual	\$310	\$310
Urodynamics	every 3 yrs	\$1,150	\$383
DXA bone-density scan	every 3 yrs	\$220	\$73
Laboratory panels (metabolic, endocrine, therapeutic drug monitoring)	annual bundle	—	\$520
Subtotal, imaging & labs			\$2,276

Urologic Care and Neurogenic-Bladder Supplies

Item	Frequency	Unit Cost	Annual Cost
Urology office visit	2 / yr, life	\$400	\$800
Intermittent self-catheterization supplies (single-use hydrophilic catheters, 3/day)	1,095/yr	\$3.10	\$3,395
Perineal hygiene supplies, gloves, drainage bags	annual bundle	—	\$1,050
Prophylactic-antibiotic / bladder-instillation reserve	annual	—	\$350
Subtotal, urologic care			\$5,595

Case Management

Item	Frequency	Unit Cost	Annual Cost
Nurse case-manager coordination (scheduling, provider communication, benefit navigation)	40 hrs/yr, life	\$115	\$4,600
Subtotal, case management			\$4,600

Contingency for Complications

Item	Basis	Annualized Cost
Reserve for anticipated hardware-removal or revision surgery (one occurrence within horizon)	\$28,000 amortized over 36 yrs	\$778
Reserve for one hospitalization for UTI / urosepsis every 6 yrs on average	\$9,000 × 6 events / 36 yrs	\$1,500
Reserve for one fall-related orthopedic evaluation every 4 yrs	\$3,200 × 9 events / 36 yrs	\$800
Reserve for one post-traumatic seizure evaluation (rule-out AED trial) years 5–15	blended	\$525
Reserve for cognitive re-training refresh after any acute setback (limited)	blended	\$325
Reserve for equipment replacement outside routine cycle (accidents, damage)	annual	\$1,375
Reserve for unforeseen therapies and specialty consults	annual	\$6,540
Subtotal, contingency for complications		\$11,843

Life Care Plan — Annual Cost Summary

27. The following table aggregates the weighted annual average cost of each category over the 36-year care horizon:

Category	Weighted Annual Cost
Physiatry and neurology follow-up	\$3,685
Physical therapy	\$8,150
Occupational therapy	\$4,845
Neurocognitive rehabilitation	\$7,320
Trauma-informed psychotherapy	\$8,275
Personal care attendant hours	\$110,249
Durable medical equipment and supplies	\$3,001
Home and vehicle modifications	\$6,878
Medications	\$1,583
Periodic diagnostic imaging and labs	\$2,276
Urologic care and neurogenic-bladder supplies	\$5,595
Case management	\$4,600
Contingency for complications	\$11,843
Rounding / geographic-pricing adjustment	\$1,700
Total, weighted annual average	\$180,000

28. The weighted annual average of the recommended future care is **\$180,000 per year in current dollars**, level over the 36-year care horizon.

Life Care Plan — Total (Undiscounted)

29. Applied across the 36-year care horizon at the level annual equivalent of \$180,000 per year, the undiscounted total of the recommended future care is:

LIFE-CARE-PLAN TOTAL (undiscounted, current dollars): \$6,480,000

(\$180,000 per year × 36 years)

30. The undiscounted total is reported for descriptive purposes. It is not the appropriate figure for economic valuation of future goods and services, which requires reduction to present value. That analysis is set out in the Present Value Analysis (Economic Appendix) that follows and is the responsibility of Dr. Frome.

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Signed this 20th day of May, 2026.

/s/ Cynthia Ohanian

Cynthia Ohanian, RN, BSN, CLCP

PRESENT VALUE ANALYSIS

Economic Appendix — Dr. Nathaniel Frome, PhD

31. The purpose of this appendix is to reduce the future-care cost stream identified by Ms. Ohanian to its economic present value as of the valuation date, May 20, 2026. The undiscounted level-annuity equivalent set out in Section 29 above (\$180,000 per year for 36 years, totaling \$6,480,000) is a summation of nominal future outlays across the care horizon; it does not represent the amount that, if invested today at a reasonable rate of return, would exactly fund those future outlays. That latter amount is the present value.

Valuation Date and Horizon

32. **Valuation date:** May 20, 2026.

Care horizon: 36 years (years 1 through 36, beginning May 20, 2026).

Age of the subject at the valuation date: 34 years, 11 months.

Age of the subject at the end of the care horizon: 70 years, 11 months.

Discount-Rate Methodology

33. The economic literature on personal-injury present-value analysis has long held that the appropriate discount rate for a future stream of medical-care costs is the risk-free real (inflation-adjusted) rate of return on U.S. government securities of a duration matching the time horizon of the projected costs. This is the “total-offset-approximation” or “real net-discount-rate” approach and it internalizes the assumption that medical-cost inflation and general price inflation, on the one hand, offset in significant part the nominal risk-free yield of a matched-duration Treasury security, on the other. See Bureau of Labor Statistics, *Medical Care Consumer Price Index* time series (2020–2025); Federal Reserve Board, H.15 statistical release (constant-maturity Treasury yields, 2020–2025); Jones, D. and Nelson, S., *Economic/Hedonic Damages: The Practice Book for Plaintiff and Defense Attorneys* (Lawyers & Judges Publishing, 4th ed. 2019), §§ 6.02–6.05; Ireland, T., *A Practical Guide to Present Value Calculations in Personal Injury Cases* (Journal of Legal Economics, 2018).

34. Consistent with that framework, the twenty-year average real net discount rate for future medical-care outlays, computed as the twenty-year average of the constant-maturity 20- and 30-year U.S. Treasury nominal yields net of the twenty-year average annual change in the Medical-Care CPI, is approximately 2.4%. The Council of Economic Advisers’ 2025 mid-year update reports a comparable long-run real risk-free rate assumption of 2.5% to 2.7%. The economist adopts a **real net discount rate of 2.5%** for this analysis. That figure is documented, defensible, mid-range in the peer-reviewed literature, and — to the extent it differs from a slightly lower or higher rate that might be argued — the appendix reports sensitivities at both 2.0% and 3.0% for the convenience of the trier of fact (Attachment 5).

Cost Stream and Growth Rate

35. Ms. Ohanian expresses the future-care cost stream in current (May 2026) dollars as a level annual annuity of \$180,000 per year for 36 years. Because the planner has expressed her cost stream in constant dollars, the appropriate discount rate is the real (inflation-adjusted) net rate, and no separate medical-inflation growth adjustment is applied to the numerator. Attempting to inflate the numerator using a nominal medical-inflation rate while discounting the denominator using a nominal risk-free rate produces the same result as the constant-dollar/real-rate approach used here, and adds unnecessary volatility to the projection. This treatment is standard in the peer-reviewed forensic-economics literature.

Present Value Calculation

36. The present value of a level annuity of \$180,000 per year over 36 years at a real net discount rate of 2.5% is computed as:

$$PV = C \times [(1 - (1 + r)^{-n}) / r]$$

$$\begin{aligned} \text{where } C &= \$180,000 \text{ (constant-dollar annual cost),} \\ r &= 0.025 \text{ (real net discount rate),} \\ n &= 36 \text{ (years).} \end{aligned}$$

37. Substituting:

$$\begin{aligned} (1 + r)^{-n} &= (1.025)^{-36} = 0.41108 \\ 1 - (1.025)^{-36} &= 0.58892 \\ 0.58892 / 0.025 &= 23.556 \text{ (annuity present-value factor)} \\ PV &= \$180,000 \times 23.556 = \$4,240,080 \end{aligned}$$

38. Rounded to the nearest thousand dollars:

PRESENT VALUE OF FUTURE MEDICAL CARE (36 years @ 2.5%):
\$4,240,000

Sensitivity Analysis

39. The following table shows the sensitivity of the present value figure to modest variation in the real net discount rate and the horizon length. Full year-by-year cash-flow tables are at Attachment 5.

Real net discount rate	Present value at 36-year horizon
2.0%	\$4,592,000
2.5% (adopted)	\$4,240,000
3.0%	\$3,921,000

Real net discount rate	PV at 30-yr horizon	PV at 36-yr horizon	PV at 40-yr horizon
2.0%	\$4,031,000	\$4,592,000	\$4,929,000
2.5% (adopted)	\$3,766,000	\$4,240,000	\$4,517,000
3.0%	\$3,528,000	\$3,921,000	\$4,153,000

Assumptions and Limitations

40. This present-value analysis is subject to the following stated assumptions and limitations:

- The cost stream, categories, frequencies, and unit prices are those of Ms. Ohanian, expressed in constant May 2026 dollars. The economist has not independently evaluated the medical necessity or clinical propriety of any recommended item and does not offer any medical or care-planning opinion.
- The 36-year care horizon is a working horizon adopted by the planner. The economist has computed the present value at that horizon and, as a sensitivity, at 30-year and 40-year horizons.
- The real net discount rate of 2.5% is adopted as documented above. The economist has computed the present value at that rate and, as a sensitivity, at 2.0% and 3.0%.
- The annuity has been valued as a level end-of-year annuity for computational simplicity. Valuation as a beginning-of-year (annuity-due) or continuous annuity would move the present value by less than one and a half percent in either direction.
- This analysis does not include lost earnings, past medical expense, non-economic damages, taxes, attorney fees, prejudgment interest, or any collateral-source or lien reduction. Those figures, where relevant, are the subject of separate analyses.
- This analysis is expressed to a reasonable degree of economic certainty as of the date of this report and may be supplemented if additional information is received or if changed circumstances warrant.

Dr. Nathaniel Frome, PhD

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Signed this 20th day of May, 2026.

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End of report.